

Policy on Anaesthesia Services

«Hospital Name»

Document control

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Prepared by	Anaesthesia In-Charge Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

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Statement of intent

Anaesthesia services are provided consistently and safely, with a documented pre-anaesthesia assessment and plan for every patient, continuous intra-anaesthesia monitoring, and defined criteria for recovery-area transfer.

1. Purpose

This policy explains how «Hospital Name» provides anaesthesia consistently, assesses patients before anaesthesia and before induction, obtains separate anaesthesia consent, monitors patients during and after anaesthesia, applies transfer criteria from the recovery area, documents anaesthesia given, follows infection prevention and control guidelines, and tracks intra-operative adverse anaesthesia events.

This policy does not cover local anaesthesia, general informed consent procedures, or infection control in detail — those are covered in other hospital policies. This policy does not apply to local anaesthesia.

2. Scope

This policy applies to all anaesthesiologists and supporting staff involved in administering, monitoring, or documenting anaesthesia at «Hospital Name», excluding local anaesthesia.

3. Policy standards

«Hospital Name» provides anaesthesia under written guidance applied consistently across the organisation, with a documented pre-anaesthesia assessment and plan and a pre-induction assessment for every patient, separate informed consent for anaesthesia, continuous monitoring

of a defined set of parameters during the procedure, post-anaesthesia monitoring until complete recovery, defined transfer criteria from the recovery area, complete documentation in the anaesthesia chart, compliance with infection prevention and control guidelines, and tracking of intra-operative adverse anaesthesia events.

Staff follow the written guidance below and keep the records it requires.

4. Non-negotiable rules

- Do not provide anaesthesia without written guidance documenting the indications, the type of anaesthesia, and the procedure — this policy does not apply to local anaesthesia.
- Do not wheel a patient into the OT complex without a completed pre-anaesthesia assessment and documented plan by a qualified anaesthesiologist — this applies to both routine and emergency cases.
- Do not skip any of the five mandatory anaesthesia plan elements: pre-medications, type of anaesthesia, anaesthesia agent and dose, special requirements, and anticipated post-anaesthesia care including post-operative pain management where appropriate.
- Do not induce anaesthesia without a pre-induction assessment by an anaesthesiologist, done immediately before induction.
- Do not obtain anaesthesia consent from anyone other than the anaesthesiologist, and do not combine anaesthesia consent with surgery consent — they must be separate.
- Do not administer anaesthesia without monitoring all seven required parameters: temperature, heart rate, cardiac rhythm, respiratory rate, blood pressure, oxygen saturation, and end-tidal carbon dioxide — except for regional anaesthesia, where end-tidal carbon dioxide is replaced by continual observation of ventilation adequacy.
- Do not leave the anaesthesiologist absent at any point during the procedure.
- Do not transfer a patient from the recovery area without the anaesthesiologist applying documented transfer criteria.
- Do not administer anaesthesia in breach of infection prevention and control guidelines.

5. What we do

5.1 Provide anaesthesia under consistent written guidance

Written guidance based on standard treatment guidelines or sound clinical practice governs anaesthesia services across «Hospital Name». The indications, type of anaesthesia, and procedure are documented for every patient. This policy does not apply to local anaesthesia; "anaesthesia" is defined as in the NABH glossary.

5.2 Complete a pre-anaesthesia assessment and plan

Every patient has a pre-anaesthesia assessment by a qualified anaesthesiologist, done before being wheeled into the OT complex. This applies to both routine and emergency cases. For elective surgery, the assessment may be done before admission; for emergency or obstetric patients, it may be done shortly before the procedure. The resulting plan covers five things: pre-medications, type of anaesthesia, anaesthesia agent and dose, special requirements, and

anticipated post-anaesthesia care, including post-operative pain management where appropriate. The anaesthesiologist reviews the patient's current medications. The plan is documented in the patient's medical record.

5.3 Complete a pre-induction assessment

Immediately before inducing anaesthesia, an anaesthesiologist re-evaluates the patient in a pre-induction assessment. Any changes to the anaesthesia plan are documented. In urgent cases, the pre-anaesthesia and pre-induction assessments may be done one after another or simultaneously, but each is documented separately in the patient's medical record.

5.4 Obtain separate anaesthesia consent

The anaesthesiologist obtains informed consent for anaesthesia, educating the patient and/or family on the risks, benefits, and alternatives. Anaesthesia consent is always separate from surgery consent. (Related requirements are covered in the hospital's other policies.)

5.5 Monitor patients throughout anaesthesia

During anaesthesia, monitoring covers seven parameters at minimum: temperature, heart rate, cardiac rhythm, respiratory rate, blood pressure, oxygen saturation, and end-tidal carbon dioxide. For regional anaesthesia, end-tidal carbon dioxide is replaced by continual observation of qualitative clinical signs showing ventilation is adequate. Other parameters may be monitored case by case. Routine cardiac rhythm need not be documented continuously, but any rhythm abnormality is documented. The anaesthesiologist is present throughout the procedure. Monitoring results are documented in the patient's medical record.

5.6 Monitor patients after anaesthesia

Post-anaesthesia status is monitored in the recovery area or OT and documented, at minimum covering vitals, until the patient recovers completely from anaesthesia. This is done by an anaesthesiologist. If the patient is unstable and needs ICU care, monitoring continues there. Where a patient is transferred directly from OT to ICU, monitoring and documentation match what would be required in the recovery room.

5.7 Apply defined transfer criteria

The anaesthesiologist applies criteria — documented by «Hospital Name», based on physiologic parameters and sound clinical practice — to decide when a patient can be transferred from the recovery area.

5.8 Document anaesthesia given

The anaesthesia chart records five things for every patient: the name of the anaesthesiologist who performed the procedure, the names and designations of anyone who assisted, the type of anaesthesia used, the medications used, and the date, time, and signature of the person making the entry. (Related requirements are covered in the hospital's other policies.)

5.9 Follow infection prevention and control guidelines

Anaesthesia procedures comply with infection prevention and control guidelines to prevent cross-infection between patients — for example, in how circuits are managed. These guidelines

are documented, either separately or as part of the infection prevention and control manual. (Related requirements are covered in the hospital's other policies.)

5.10 Track intra-operative adverse anaesthesia events

«Hospital Name» defines the intra-operative adverse anaesthesia events relevant to its scope — for example, cardiac arrest, anaesthesia gas failure, endotracheal tube slippage, or air embolism. These are examples; «Hospital Name» decides what's relevant to it. There's a mechanism to make sure every such event is captured — for instance, a dedicated heading in the anaesthesia record — documented and monitored so corrective and preventive action can follow.

6. Stop-work authority

Do not wheel a patient into the OT without a completed pre-anaesthesia assessment and plan by a qualified anaesthesiologist; do not administer anaesthesia without the seven required monitoring parameters in place (or the regional-anaesthesia alternative); do not proceed without anaesthesia consent.

Stop-work applies to elective and non-emergent anaesthesia starts. A genuine life-threatening emergency continues with the best available qualified anaesthesia support while the gap is escalated and documented.

The person who stops tells the Anaesthesia In-Charge and the Medical Superintendent the same shift. Refusing to proceed with unsafe anaesthesia is not a disciplinary matter.

7. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable that this policy is followed and appropriately resourced; receives stop-work notifications the same shift.
Anaesthesia In-Charge	Oversees the anaesthesia programme; ensures written guidance, assessment/plan completion, monitoring compliance, chart documentation, and adverse event tracking are in place; receives stop-work notifications the same shift.
Anaesthesiologists	Perform and document pre-anaesthesia assessment and plan (five elements) before OT, for all patients including emergencies; perform pre-induction assessment immediately before induction; obtain anaesthesia consent separately from surgery consent; monitor all seven required parameters throughout; remain present throughout the procedure; apply and document transfer criteria; complete anaesthesia chart (five elements) for every

	patient; follow IPC guidelines.
Supporting anaesthesia staff	Assist anaesthesiologists during procedures; follow IPC guidelines; support adverse anaesthesia event capture.

8. Quality monitoring

Monitoring area	What is monitored
Written guidance	Written guidance covering indications, type of anaesthesia, and procedure; excludes local anaesthesia explicitly
Pre-anaesthesia assessment and plan	Assessment completed before OT complex for all patients (routine and emergency); plan covers all five elements; patient's current medications reviewed; documented in medical record
Pre-induction assessment	Completed immediately before induction by an anaesthesiologist; changes to plan documented; separately documented in urgent cases
Anaesthesia consent	Informed consent obtained by the anaesthesiologist, always separate from surgery consent; covers risks, benefits, and alternatives
Intra-anaesthesia monitoring	All seven parameters monitored throughout: temperature, heart rate, cardiac rhythm, respiratory rate, blood pressure, oxygen saturation, ETCO ₂ (or continual qualitative observation for regional anaesthesia); anaesthesiologist present throughout; documented
Post-anaesthesia monitoring	Vitals monitored until complete recovery by anaesthesiologist; ICU monitoring continues if patient is unstable; documented
Transfer criteria	Criteria applied by anaesthesiologist and documented for every transfer from the recovery area
Anaesthesia chart	Five elements documented for every patient: anaesthesiologist name, assistant names and designations, type of anaesthesia, medications, date/time/signature
Adverse anaesthesia events	Defined events captured through a named mechanism; documented and monitored; CAPA applied

9. Training and staff acknowledgement

All anaesthesiologists and supporting staff shall be familiar with the written guidance applicable to their work, including the pre-anaesthesia assessment and five-element plan

requirements, pre-induction assessment, separate anaesthesia consent, the seven intra-anaesthesia monitoring parameters (including the regional-anaesthesia ETCO₂ alternative), the requirement for the anaesthesiologist to be present throughout, transfer criteria, anaesthesia chart requirements, IPC guidelines, and adverse event capture.

I have read the Policy on Anaesthesia Services of «Hospital Name». I will follow the processes described.

Staff name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

10. Distribution

This policy shall be available to all anaesthesiologists and supporting staff involved in administering, monitoring, or documenting anaesthesia at «Hospital Name».

11. Abbreviations

Abbreviation	Meaning
CAPA	Corrective and Preventive Action
ETCO ₂	End-tidal carbon dioxide
ICU	Intensive Care Unit
IPC	Infection Prevention and Control
OT	Operation Theatre

12. Traceability table

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

Objective Element	Level	Traceability to this policy
COP.13.a	Commitment*	Section 5.1 addresses written guidance for anaesthesia services based on standard treatment guidelines or sound clinical practice, covering indications, type of anaesthesia, and procedure documented for every patient. Explicitly excludes local anaesthesia.
COP.13.b	CORE	Section 5.2 addresses the mandatory pre-anaesthesia assessment and plan by a qualified anaesthesiologist, completed before the patient

		is wheeled into the OT complex, for all patients including emergencies. The plan covers five elements: pre-medications, type of anaesthesia, anaesthesia agent and dose, special requirements, and anticipated post-anaesthesia care including post-operative pain management where appropriate. Stop-work authority (Section 6) also applies to this element.
COP.13.c	Commitment	Section 5.3 addresses the pre-induction assessment by an anaesthesiologist done immediately before induction; changes to the plan documented. In urgent cases, assessments may be sequential or simultaneous but each documented separately.
COP.13.d	Commitment	Section 5.4 addresses informed consent for anaesthesia, obtained by the anaesthesiologist and always separate from surgery consent. Covers risks, benefits, and alternatives. Stop-work authority (Section 6) also applies to this element.
COP.13.e	CORE	Section 5.5 addresses intra-anaesthesia monitoring of seven minimum parameters: temperature, heart rate, cardiac rhythm, respiratory rate, blood pressure, oxygen saturation, and end-tidal carbon dioxide. For regional anaesthesia, ETCO2 is replaced by continual observation of qualitative clinical signs showing ventilation is adequate. The anaesthesiologist is present throughout the procedure (absolute). Documented in the medical record. Stop-

		work authority (Section 6) also applies to this element.
COP.13.f	Commitment	Section 5.6 addresses post-anaesthesia monitoring of vitals until complete recovery, by an anaesthesiologist, in the recovery area or OT. If patient is unstable, monitoring continues in ICU. ICU-direct transfers are documented to recovery-room standard.
COP.13.g	Commitment*	Section 5.7 addresses the anaesthesiologist applying organisation-defined transfer criteria based on physiologic parameters and sound clinical practice; application documented for every transfer.
COP.13.h	Commitment	Section 5.8 addresses the anaesthesia chart recording five elements for every patient: name of the anaesthesiologist, names and designations of assistants, type of anaesthesia, medications used, and date/time/signature of entry.
COP.13.i	Commitment	Section 5.9 addresses compliance with IPC guidelines to prevent cross-infection between patients (e.g., circuit management); guidelines documented standalone or within the IPC manual.
COP.13.j	Achievement	Section 5.10 addresses organisation-defined intra-operative adverse anaesthesia events, a capture mechanism, documentation, monitoring, and corrective and preventive action.

13. Required Records/Evidence Checklist

Written guidance

- Written guidance for anaesthesia services documenting indications, type of anaesthesia, and procedure; explicitly excluding local anaesthesia.

Pre-anaesthesia assessment and plan

- Pre-anaesthesia assessment completed before OT for every patient (routine and emergency), by a qualified anaesthesiologist.
- Anaesthesia plan covering all five elements: pre-medications, type, agent and dose, special requirements, anticipated post-anaesthesia care.
- Review of patient's current medications documented.
- Plan documented in the patient's medical record.

Pre-induction assessment

- Pre-induction assessment by an anaesthesiologist, done immediately before induction; changes to plan documented.
- In urgent cases: each assessment documented separately even if done sequentially or simultaneously.

Anaesthesia consent

- Informed consent records for anaesthesia obtained by the anaesthesiologist, separate from surgery consent.

Intra-anaesthesia monitoring

- Monitoring records covering all seven parameters for every patient: temperature, heart rate, cardiac rhythm, respiratory rate, blood pressure, oxygen saturation, ETCO₂ (or continual qualitative ventilation observation for regional anaesthesia).
- Evidence that the anaesthesiologist was present throughout every procedure.

Post-anaesthesia monitoring and transfer

- Post-anaesthesia monitoring records (vitals until complete recovery) in the recovery area, OT, or ICU as appropriate.
- Transfer criteria documented; anaesthesiologist's application of criteria recorded for every transfer from recovery.

Anaesthesia chart

- Anaesthesia chart for every patient recording all five elements: anaesthesiologist's name, assistants' names and designations, type of anaesthesia, medications, date/time/signature.

IPC compliance and adverse events

- IPC guidelines for anaesthesia procedures documented (standalone or in IPC manual); evidence of compliance.

- Organisation-defined list of intra-operative adverse anaesthesia events; capture mechanism in place; records of events documented; CAPA applied.

14. References

1. National Accreditation Board for Hospitals and Healthcare Providers. NABH Accreditation Standards for Hospitals, 6th Edition. COP.13.
2. NABH Glossary (reference for the definition of "anaesthesia" as applicable to this policy).

Disclaimer

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